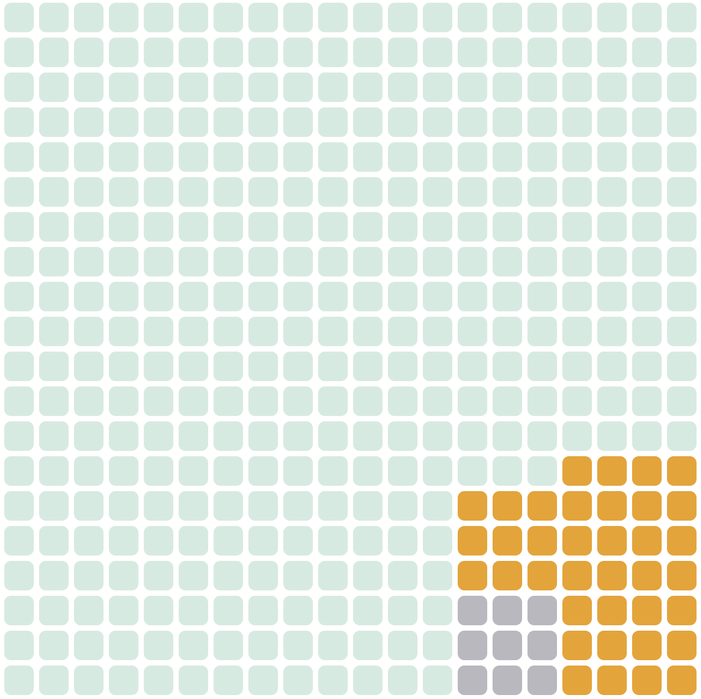
ClaimBack AI agents that appeal insurance denials

Most appeals win.
Almost nobody appeals.



Never appealed
The insurer's decision stands.

11.5% appealed

81% of appeals win
fully or partially

The rest lose

Insurers deny at machine speed.
Patients appeal by hand.

1.2 seconds for the insurer to deny

> At this scale, 1.2 seconds wouldn't even be visible.

Cigna doctors, as reported by ProPublica. Cigna disputes this.

~6 months to fight it

Round 1	45–60 days
Round 2	45–60 days
Round 3	45–60 days

Fighting denials is a $19.7B-a-year job.

450M
denied claims a year

×

$43.84
to fight each one

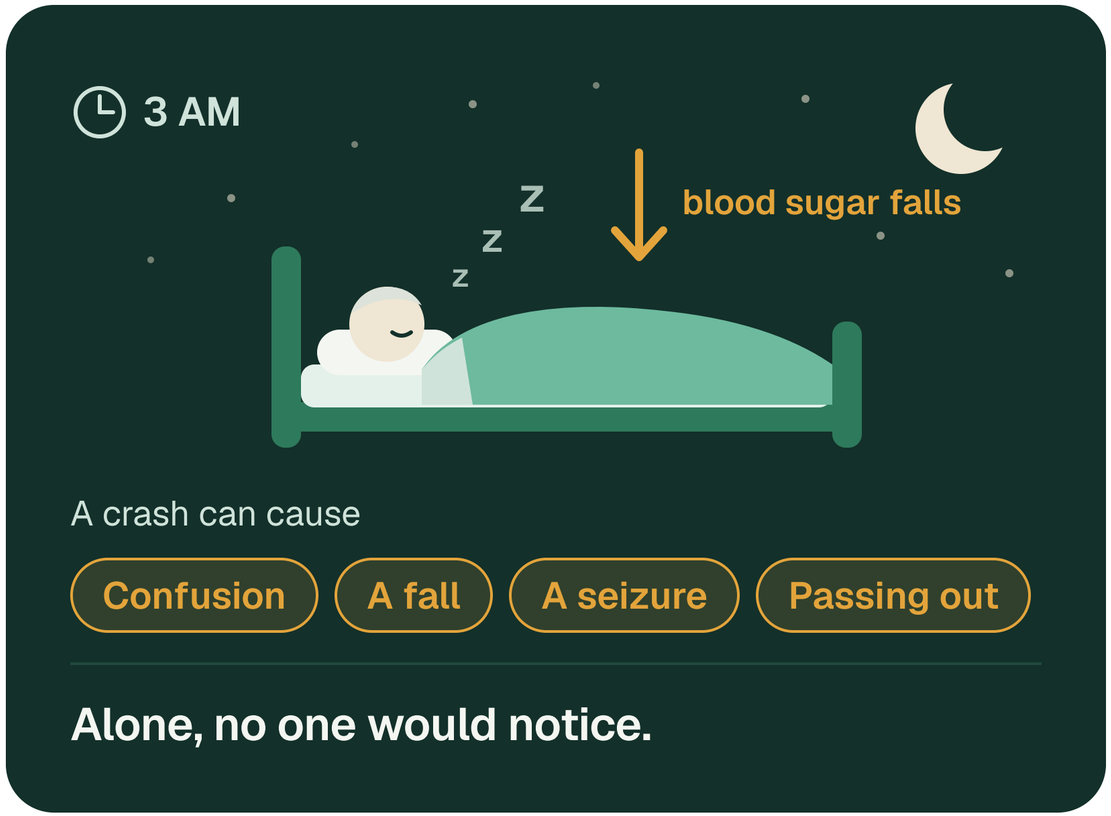
=

$19.7B
spent by providers

Hospitals pay teams to fight. Patients fight alone.

Rosa’s blood sugar crashes at night.

65 · type 2 diabetes · **lives alone**



Her doctor

Prescribes a glucose monitor

its alarm wakes her when she goes low

Her insurer

Denied

Rule: 3+ insulin shots a day.

Rosa takes 1.

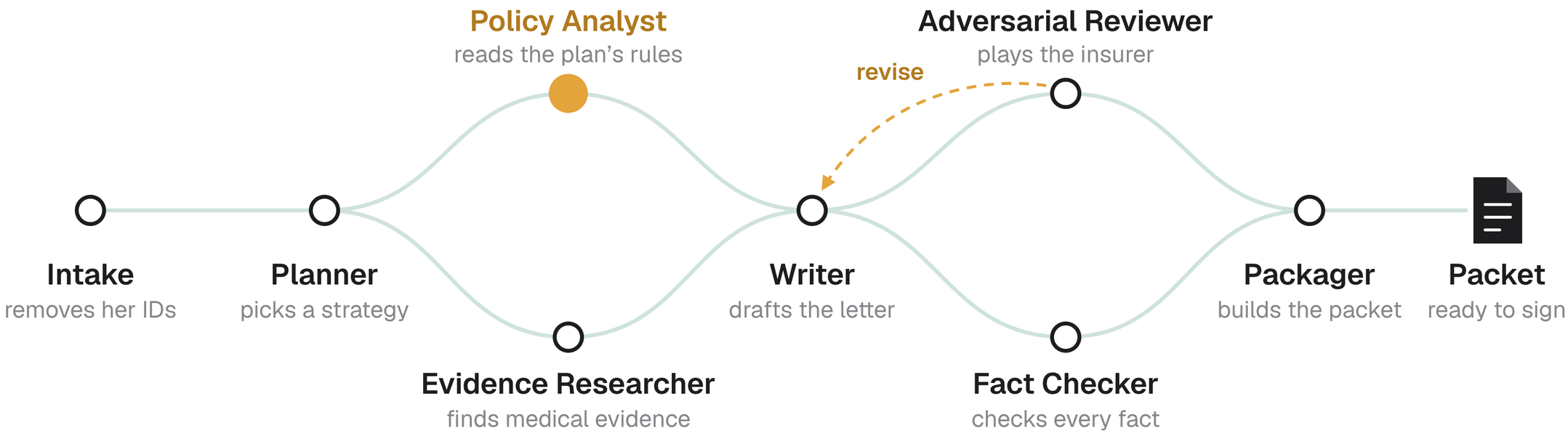
The catch

That rule no longer exists.

Her insurer removed it in January.

ClaimBack does the fighting.

Seven Claude agents take Rosa's case.



Policy Analyst: the denial cites a rule the plan removed on 01/01/2026.

One agent plays the insurer.

It tries to reject the letter before the real insurer can.

Step 1

Draft 1

Writer drafts the appeal

→

Step 2 · the insurer's side

4 weak spots

> The letter never names the glucose monitor.

Sent back

→

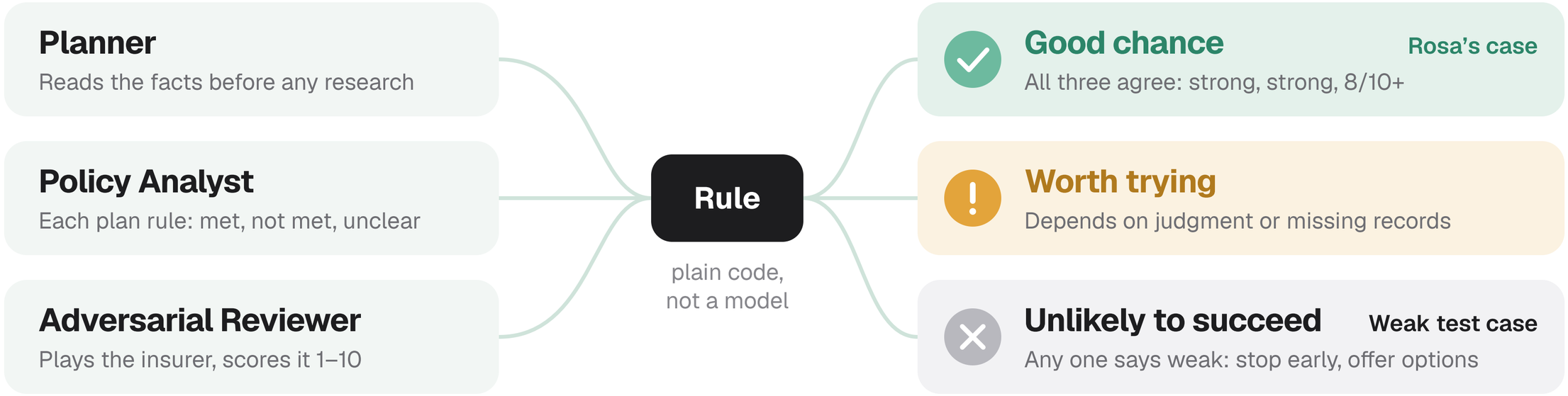
Step 3

Draft 2 approved

Every fact checked against her records and the policy

Three judges rate every appeal.

All three must agree to say **“Good chance”**. Any one can say **“Unlikely”**.



Every rule and medical fact the judges use is checked against a primary source

Federal appeal rules	Medicare CGM criteria	ADA Standards of Care	ACR imaging criteria
29 CFR 2560.503-1	LCD L33822	2026 edition	Low back pain

What Rosa gets: an appeal ready to sign.

Catches the insurer’s mistake

The rule they cited was dropped in January

Appeal due March 21, 2027

180 days after the denial

MN-04: Denied as not medically necessary: insurer states MP-DME-031 requires intensive insulin therapy (3+ daily injections or insulin pump), and member uses only once-daily long-acting insulin. Note: the provided policy document shows this requirement was removed in the 01/01/2026 revision; current criterion I.B.1 covers any insulin regimen including single daily basal insulin, and I.B.2 covers documented problematic hypoglycemia.

The insurer’s own policy dropped this rule · real output, highlight added

It keeps working the case for weeks.

It watches the inbox and acts on every reply.

Day 0

Appeal sent

Rosa approves the send

Day 3

Insurer: “received”

Not a decision, so it keeps waiting

Day 8

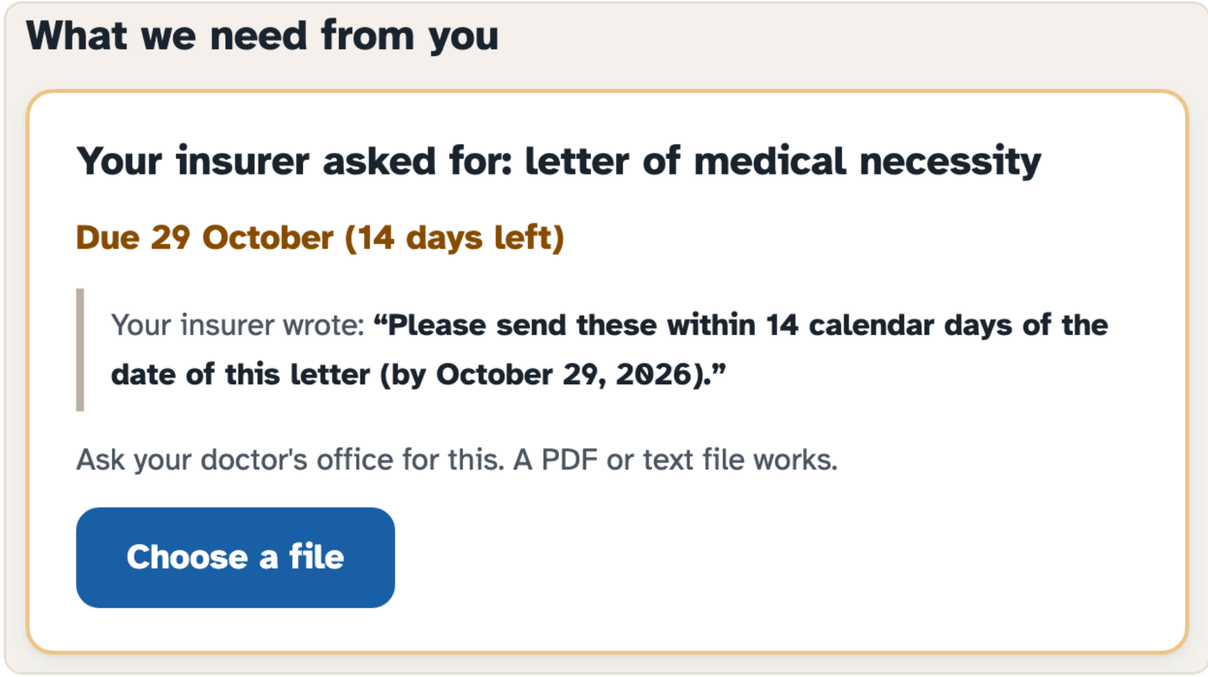
Insurer asks for evidence

A task for Rosa, due in 14 days

Later

Decision arrives

Closes only when Rosa confirms



The patient app on Rosa’s test case · the insurer’s emails are simulated

Tested live. About $3 an appeal.

✓	Rosa’s case **42 of 42 facts verified, survived a crash**	**$2.80** ~10 min
✓	A new denial type (MRI) **Found the exception the insurer ignored**	**$2.60** 10.8 min
✓	A planted fake hospitalization **Flagged it and invented nothing**	**$3.10** 9.1 min
✓	A denial that was correct **Said “weak case” and stopped early**	**$1.39** 4.7 min

Paid only when an appeal wins.

Free merit check
weak cases stop early

Appeal + casework
letters, replies, deadlines

Success fee
only after a confirmed win

Fee rate not set yet · alternatives: Claimable $39.95 per appeal, Counterforce and Fight Health Insurance free, Waystar enterprise

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The agents do the work. People decide.

ClaimBack